

# MEETING VETERANS WHERE THEY ALREADY ARE: A TRIAL OF APP-BASED, COMMUNITY-EMBEDDED SUICIDE PREVENTION

**Author:** Terry D. Flood, DHA, DBA candidate

**Program:** Doctor of Health Administration (DHA), Implementation Science - Dartmouth / Geisel School of  
Medicine

**Course:** IS 751-753 MSIS Capstone Impact Project

**Assignment:** Spring Assignment 6 of 8 Final Report

**Defense Date:** June 3, 2026

**Project Type:** Implementation Plan

---

### ***Author's Note (Positionality)***

I came to this work the long way. Seventeen years in uniform - 2002 to 2019. Six years as a Senior Trainer for the Department of Veterans Affairs' Veterans Crisis Line (988), where the calls that taught me the most were the ones where the caller said, "I almost didn't pick up the phone." Here is how and why this is personal to me and should resonate with the reader. After 17 years of service, I was suddenly forced to medically retire due to the discovery by the military that I was diagnosed with multiple sclerosis in 2008. I had flourished in the service, 7 promotions in 7 years, went from enlisted Private, to 1sg E-8, in 10 years, 3 combat deployments totaling 40 months, all to E. Paktika Province Afghanistan, suffered two traumatic brain injuries to rocket explosions, lost many comrades, but fortunately never lost any service-member under my direct supervision. I came home from the deployments different every time. I suffered morale injury, misdiagnosed as PTSD. I couldn't talk to anyone about the experience, everyone was either deceased, dealing with their own issues, or seemed to lack the capability to understand process, so I internalized.

I planned on serving for 30 years. I went from senior enlisted to senior Warrant Officer, targeting was my job. In four months, my life changed completely: forced retirement, a family move, a search for new work, a diagnosis I had long ignored, painful memories, and a difficult transition into civilian life.

The transition began badly. I was hired by the VA in a new city, but I had no support system or local connections. My wife said she could not handle any of it and left, making me the custodial father of four children, ages 9 to 16. My family blamed me for the divorce and for her leaving the children. I hit rock bottom. Even though I knew about 988, when I called and heard the operator's tone, I hung up before saying a word. One afternoon at Belton Lake, Texas, I sat in my truck calculating whether anyone would notice if I were gone and how fast I would need to drive into the lake to make my death look accidental so the insurance policy would care for my children.

Then a family member sent an unexpected text. It was my sister. She said, in effect, "You've been quiet, and that means you are either very busy or about to do something very foolish." That text, and the call that followed, was the right intervention at the right time, even though neither was intended as a clinical tool. I had never experienced suicidal ideation before. Research suggests that many suicidal crises last only 10 minutes to an hour, and the military-to-civilian transition is one of the most difficult periods a service member can face.

It was a relationship doing its job. I am writing this plan because most of the veterans the VA's own 2023 annual report describes—the 57.3% not connected to VA care at the time of death—will never sit in the chair I sat in for six years on the Crisis Line. They will be sitting in the driver's seat I sat in at Belton Lake, or in a closed garage with the engine running, or in a room alone with their firearm and catastrophic thoughts. The question this Implementation Plan asks is whether we can build an architecture that puts a trusted person, equipped with validated tools, inside those trucks, at those lakes, before the ten-minute deliberation window closes. In the moment, the brain doesn't allow you to be future focused, you just want the suffering to end, to stop being a burden to others.

8 years later, I am a grandfather of four, my children are doing well, and I remarried. As an aside I went on to earn a doctorate in healthcare & business administration, and this will be my 5th Master of Science degree. I took that situation and was able to get connected to sustainable support and have lived a purpose driven life ever since. So, I feel it is my duty to pay it forward. I have established a non-profit called thriving

communities and develop tools and connect with agencies such as the United Way to reach marginalized populations in an empathetic and engaging way, while using technology to monitor outcomes.

That story is a living example of the population-level problem this trial is designed to address. It is not the evidentiary basis of this plan. The evidence base is the peer-reviewed implementation-science literature summarized in §2 and the validated instruments specified in §4.5. The personal story appears here so that the design choices in §§3-5 are legible to the reader as something other than abstractions; the design choices themselves are defended on methodological rigor, not on biography. Veterans are not monolithic—what worked for one veteran at one lake on one afternoon is one observation, not a population claim. The trial is structured to learn what generalizes across the population, with pre-registered hypotheses, blinded estimation, and a pre-specified termination region (§6.4).

## Abstract

**The counterfactual:** Approximately 57.3% of U.S. veterans who die by suicide are not connected to Department of Veterans Affairs (VA) health care or benefits at the time of death (U.S. Department of Veterans Affairs, 2023). Validated suicide-risk detection and response interventions exist but do not reach this population because they live inside clinical systems veterans do not enter until they are already in crisis. This Implementation Plan defines a Type-1 effectiveness-implementation hybrid randomized trial of the Community-Based Behavioral Health Engagement Model (CBHEM) delivered through the M2C Transition platform (Mission to Civilian Transition; [vetmissiontransition.com](http://vetmissiontransition.com)) - a live Beta technology platform built by the author and operated as a nonprofit - embedded in five veteran-serving community organizations across three states plus a digital-only recruitment cohort. Veterans not currently engaged with VA mental-health care are individually randomized 1:1 to CBHEM-via-M2C with trained champion support or to usual peer support as currently delivered. The primary outcome is the change in Suicide Behaviors Questionnaire-Revised (SBQ-R) total score from baseline to 6 months. Power is set at 0.90 against Cohen's  $d = 0.30$ ,  $\alpha = 0.05$  two-sided, yielding  $n = 720$  enrolled,  $n = 468$  analyzable under a 35% attrition assumption anchored to Owen et al. (2020) and Lattie et al. (2022). The implementation aim (Aim 2) evaluates eight Proctor et al. (2011) outcomes mapped to the RE-AIM framework (Glasgow et al., 1999), integrated quantitatively and qualitatively through joint displays (Fetters et al., 2013). The analytic plan is pre-registered on the Open Science Framework before enrollment and includes a pre-specified termination region. If the trial supports CBHEM, it will produce the first randomized evidence that an embedded community-organization architecture can deliver validated suicide-risk detection and response to the population the VA's own data identifies as the population dying.

## 1. Problem Statement

Veterans transitioning out of military service experience disruptions of identity, structure, and belonging that place them at heightened risk for mental health conditions, employment instability, and suicide. Department of Veterans Affairs clinical services are effective for veterans who reach them. Still, the 2023 VA National Veteran Suicide Prevention Annual Report documented that 57.3% of veteran suicides in 2023 occurred among veterans who were not connected to VA health care or benefits at the time of death (U.S. Department of Veterans Affairs,

2023, p. iv). The age- and sex-adjusted veteran suicide rate is approximately 1.5 to 2.4 times the rate among non-veteran U.S. adults. This is not a discovery problem - evidence-based interventions exist; it is an implementation problem (Bauer & Kirchner, 2020). Validated interventions do not reach veterans in the everyday community, workplace, and family environments where distress accumulates before crisis.

A gap exists between early-warning signs of mental health distress among veterans and system-level interventions. The gap emerges in settings outside traditional health care—community organizations, workplaces, and family relationships—and is sustained by the absence of an infrastructure that pairs trusted relational access with systematic assessment, signal tracking, and adaptive response. This Implementation Plan operationalizes the Community-Based Behavioral Health Engagement Model (CBHEM) through the M2C Transition platform - a live Beta technology platform built by the author and operated as a nonprofit - deployed across five veteran-serving community organizations in three states and a digital-only cohort, in a 12-month Phase 1 randomized trial.

## 2. Use of Evidence

---

The Implementation Plan rests on three converging bodies of peer-reviewed evidence, fully developed in the accompanying literature review (Flood, 2026) and summarized here for methodological coherence. First, transition distress is a psychological stressor distinct from combat exposure. Mobbs and Bonanno (2018), Bergman et al. (2020), and Romero et al. (2020) established that loss of structure, mission, and belonging—not deployment trauma alone—drives transition difficulty. Bryan et al. (2020) extended this to suicidal ideation; Runeson et al. (2019) added population-level evidence linking job loss to suicide; Smith et al. (2021) showed that resilience erodes under occupational and social stressors without targeted reinforcement.

Second, detection-and-response failures stem from timing issues. Bauer and Kirchner (2020) document a 17-year evidence-to-practice delay; Deisenhammer et al. (2009) found that 48% of survivors of near-lethal suicide attempts described a deliberation window of less than ten minutes, and 24% described a window of less than five minutes. Simon et al. (2001) replicated this finding in firearms attempts. Standalone digital tools cannot bridge this gap: Mohr et al. (2019) document the efficacy-effectiveness collapse, Owen et al. (2020) found that veteran mHealth engagement drops within weeks without peer support, and Lattie et al. (2022) report 50-80% attrition for public-facing digital mental health platforms. Technology embedded inside trusted relational networks is the only configuration that has held up empirically.

Third, community-based organizations are the missing infrastructure. Currier et al. (2019) demonstrated peer-support effectiveness for PTSD and suicidal ideation in veterans. Burnett-Zeigler et al. (2019) established the value of community participation in digital mental health design. Watkins et al. (2021) identified the absence of literature on community-level engagement of veterans not connected to VA care as a field-level gap. The CBHEM responds to that gap by embedding M2C Transition inside veteran-serving community organizations and instrumenting the relational moment of "someone checked on me" as a designed, repeatable activity.

All methodological claims in this Implementation Plan are substantiated by peer-reviewed primary sources. Supporting references - limited to federal evaluation reports (U.S. Department of Veterans Affairs, U.S. Department of Agriculture Economic Research Service) and the validated PCL-5 instrument manual (Weathers et al., 2013) - are used only for empirical anchoring (e.g., the 57.3% population statistic), geographic stratification (RUCA tier definitions), and instrument administration. No anecdotal or non-academic sources are used.

### 3. Theoretical, Model, and Framework Stack

This Implementation Plan uses two primary theoretical models and frameworks (TMFs) and four complementary measurement and design instruments. The two primary TMFs are the Consolidated Framework for Implementation Research (CFIR; Damschroder et al., 2009), which structures contextual assessment across outer setting, inner setting, characteristics of individuals, intervention characteristics, and implementation process; and the Reach, Effectiveness, Adoption, Implementation, and Maintenance framework (RE-AIM; Glasgow et al., 1999), which structures the evaluation design across five dimensions spanning individual-level effect and system-level penetration.

Four complementary instruments sit underneath the primary TMFs. The Proctor et al. (2011) Implementation Outcomes taxonomy operationalizes the RE-AIM Implementation dimension through eight measurable outcomes (acceptability, adoption, appropriateness, feasibility, fidelity, implementation cost, penetration, sustainability). Moore et al. (2009) seven-level outcomes hierarchy operationalizes evaluation of champion training, an essential layer because CBHEM depends on champion competence. Curran et al. (2012) hybrid-typology defines the Type-1 effectiveness-implementation design used here. Aarons et al. (2017) scaling-out theory guides the Phase 2 and Phase 3 expansions.

**Table 1. TMF stack used in this Implementation Plan**

| Element                                | Type                       | Function                                                                                                           | Citation                  |
|----------------------------------------|----------------------------|--------------------------------------------------------------------------------------------------------------------|---------------------------|
| <b>CFIR</b>                            | Primary TMF (determinants) | Structures readiness assessment; identifies barriers and facilitators; informs choice of implementation strategies | Damschroder et al. (2009) |
| <b>RE-AIM</b>                          | Primary TMF (evaluation)   | Structures evaluation across Reach, Effectiveness, Adoption, Implementation, Maintenance                           | Glasgow et al. (1999)     |
| <b>Proctor Implementation Outcomes</b> | Measurement instrument     | Operationalizes the RE-AIM Implementation dimension via 8 outcomes                                                 | Proctor et al. (2011)     |
| <b>Moore's Evaluation Framework</b>    | Measurement instrument     | Evaluates champion training across 7 levels                                                                        | Moore et al. (2009)       |
| <b>Curran et al. hybrid typology</b>   | Design framework           | Defines the Type-1 effectiveness-implementation design                                                             | Curran et al. (2012)      |
| <b>Aarons scaling-out theory</b>       | Adjunct framework          | Guides Phase 2/3 expansion to new populations and delivery systems                                                 | Aarons et al. (2017)      |

## 4. Implementation Plan

---

### 4.1 Study Design

The Phase 1 trial is a Type-1 effectiveness-implementation hybrid (Curran et al., 2012) with effectiveness as the primary aim and implementation outcomes as the secondary aim. The unit of randomization is the veteran. Permuted-block randomization (block size 4) is stratified on three covariates with established prognostic value for veteran suicidality - sex/gender (2 levels), age category (3 levels: 18-34, 35-54, 55+), and Rural-Urban Commuting Area tier (4 levels) (USDA Economic Research Service, n.d.) - producing 24 strata. Allocation is concealed through a centralized REDCap module. Site (5 levels across three states: Texas x 2, Mountain West x 1, Northeast x 1, digital-cohort x 1) enters the analytic model as a fixed effect; champion (nested within site) enters as a random intercept. The trial is open-label by necessity; the statistician computing the primary outcome is blinded to assignment until the pre-registered analysis is locked.

The design is mixed-methods and convergent. Quantitative data (validated self-assessment scores, platform telemetry, champion training scores, and CFIR-based readiness scores) are collected on a fixed cadence. Qualitative data (semi-structured veteran interviews) are collected at 3 and 12 months. Quantitative and qualitative streams are integrated during analysis using a joint-display approach (Fetters et al., 2013).

### 4.2 Selection Criteria

#### **Pilot organization selection criteria:**

- **Inclusion:** Operates as a veteran-serving community organization (501(c)(3) or equivalent). Serves at least 100 unique veterans per quarter at the point of screening. Has at least one identifiable internal champion willing to commit to platform onboarding, training, and a 12-month engagement. Possesses, or can secure within 60 days, basic data-handling capacity (secure internet, designated data steward, and HIPAA-aligned policies). The site footprint spans the urban / suburban / rural distribution across three states (Texas x 2 - urban and suburban-rural; one Mountain West rural; one Northeast suburban) plus a digital-only cohort.
- **Exclusion:** Existing exclusive partnership with a competing platform that prohibits adoption of M2C Transition during the pilot window. Active major organizational restructuring (e.g., merger, leadership transition within 90 days of onboarding) that would confound the CFIR inner-setting assessment.

#### **Veteran participant selection criteria:**

- **Inclusion:** Age  $\geq 18$ . Verified veteran status (DD-214 or equivalent). SBQ-R total  $\geq 4$  or C-SSRS Ideation Severity  $\geq 1$  at screening. Able to consent in English. Access to a phone or platform-compatible device for follow-up contact. Recruited through a Phase 1 channel (CBO direct, Texas Veterans Commission referral, American Red Cross Military-2-Veteran Network consortium, Team RWB, or digital cohort).
- **Exclusion:** Active engagement with VA mental-health services in the prior 90 days (preserves the target population). Acute psychiatric instability requires inpatient referral at screening (such veterans are routed to clinical care via the safety-net pathway and are not randomized). Inability to provide consent.

### 4.3 Readiness-for-Change Assessment

Each pilot organization completes the CBHEM Organizational Readiness Assessment (Appendix A) before onboarding. The instrument is constructed from the CFIR inner-setting domain and includes 18 items across five subdomains: leadership engagement, available resources, access to knowledge and information, networks and

communications, and tension for change. Items use a 1-5 Likert scale; subdomain scores are computed as means; an organization-level composite below 3.0 triggers a structured conversation to adapt the implementation strategy before onboarding proceeds. The instrument is administered at three time points: baseline (T0, pre-onboarding), six months (T1), and twelve months (T2). Change scores at the subdomain level are reported in the Phase 1 evaluation deliverable as part of the CFIR inner-setting analysis.

#### ***4.4 Stakeholder Engagement Plan***

Stakeholder engagement follows the Stakeholder Engagement Matrix (Appendix B), grouped into three functional tiers.

- **Tier 1 — Internal accountable:** Organizational leadership at each pilot site, engaged quarterly via a structured progress review; site champions, engaged biweekly via a 30-minute implementation huddle and continuously through platform-supported workflow.
- **Tier 2 — Lived-experience governance:** A Veteran Advisory Group of six to nine veterans drawn from across the five pilot sites and the digital cohort, engaged monthly to review platform changes, instrument language, and emerging adaptation needs.
- **Tier 3 — External system bridges:** State-level liaisons in each represented state (Texas Veterans Commission, Mountain West state veterans coordinator, Northeast state veterans coordinator), a VA primary-care liaison, the American Red Cross Military-2-Veteran Network coordinating partner, and a Team RWB chapter representative for the digital cohort, engaged quarterly to support warm handoffs to formal care; plus the IRB and data-steward function engaged as needed for protocol and consent decisions.

Engagement decisions are documented through a lightweight memo log maintained by the project lead, with monthly summaries reviewed by the Veteran Advisory Group. Where the Veteran Advisory Group requests changes to platform language, instrument phrasing, or pathway logic, those changes are tracked as protocol deviations, version-controlled in the platform release notes, and reported in the final evaluation as evidence of CFIR-aligned adaptation (Aarons et al., 2017).

#### ***4.5 Instruments and Data Collection***

Six instruments are used in Phase 1. Each is reproduced in full in the appendices.

**Table 2. Instruments, constructs, cadence, and data source**

| <b>Instrument</b>                                    | <b>Construct</b>                                      | <b>Cadence</b>                                                 | <b>Source / Appendix</b>            |
|------------------------------------------------------|-------------------------------------------------------|----------------------------------------------------------------|-------------------------------------|
| <b>SBQ-R (primary outcome)</b>                       | Suicidal behaviors                                    | Baseline, 3 mo, 6 mo                                           | Osman et al. (2001); Appendix H     |
| <b>C-SSRS (confirmatory clinical-grade endpoint)</b> | Suicide ideation severity + behavior                  | Baseline, 6 mo, triggered when SBQ-R $\geq 7$                  | Posner et al. (2011)                |
| <b>PHQ-9 (secondary)</b>                             | Depressive symptom severity                           | Baseline, 3 mo, 6 mo, triggered by Yellow tier                 | Kroenke et al. (2001); Appendix C   |
| <b>WHO-5 (secondary)</b>                             | General well-being                                    | Baseline, 3 mo, 6 mo + monthly drop-in                         | Topp et al. (2015); Appendix E      |
| <b>PCL-5 (triggered, descriptive)</b>                | PTSD symptom severity                                 | Baseline; triggered on Yellow-tier with PTSD-dominant judgment | Weathers et al. (2013); Appendix D  |
| <b>CBHEM Organizational Readiness</b>                | CFIR inner-setting readiness                          | T0, T1 (6 mo), T2 (12 mo)                                      | Author, CFIR-derived; Appendix A    |
| <b>Champion Training Pre/Post Survey</b>             | Moore Levels 1-4                                      | Pre-training, immediately post, 90-day follow-up               | Moore et al. (2009); Appendix F     |
| <b>Semi-Structured Veteran Interview Guide</b>       | Acceptability, appropriateness, feasibility (Proctor) | 3 mo and 12 mo (purposive sample $n \approx 30$ )              | Author, Proctor-aligned; Appendix G |

Platform telemetry collected automatically by M2C Transition includes registration timestamps, check-in frequency, self-assessment completion rates, tier transitions (Green / Yellow / Red), Safety Plan creation events, 988 page interactions, VSO Directory clicks, and Mentorship and Community Room activity. All telemetry is stored in a HIPAA-aligned, encrypted backend; identifiers are stripped from any analytic export.



## 4.6 Procedures and Timeline

**Table 3. Phase 1 (12-month) procedures and timeline**

| Months                         | Activities                                                                                                                                                                                                 | Sites Involved      |
|--------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------|
| <b>Pre-launch<br/>(-2 → 0)</b> | Site recruitment; CBHEM Organizational Readiness (T0); IRB and data-use agreements; champion identification; M2C Transition tenant provisioning; OSF pre-registration                                      | All                 |
| <b>1–2</b>                     | Sites A, B, C onboarding (Texas x 2 + Mountain West); champion training cohort 1; veteran enrollment opens at A, B, C; Veteran Advisory Group constituted                                                  | A, B, C             |
| <b>3–4</b>                     | Site D onboarding (Northeast); digital-cohort recruitment open via Red Cross M2VN consortium and Team RWB chapter network; champion training cohort 2                                                      | A, B, C, D, digital |
| <b>5–6</b>                     | Steady-state enrollment continues; 3-month SBQ-R/PHQ-9/ WHO-5 waves at A, B, C; first qualitative interview wave (3-mo follow-up); T1 readiness at A, B, C                                                 | All                 |
| <b>7–9</b>                     | Steady-state operations; biweekly champion huddles; monthly Veteran Advisory Group; monthly safety review; 6-month primary-outcome wave at A, B, C; C-SSRS confirmatory administration at the 6-month wave | All                 |
| <b>10–12</b>                   | 6-month wave at D and digital cohort; T2 readiness; second qualitative interview wave (12-mo); Phase 1 evaluation report drafted                                                                           | All                 |

## 4.7 Safety, Ethics, and Risk Management

All Yellow-tier signals trigger champion outreach within 72 hours; all Red-tier signals trigger champion outreach within 24 hours and platform-initiated 988 access on the user device. The platform displays 988 access on every page and surfaces the in-platform Safety Plan immediately on Red-tier triage. Site champions complete a platform-administered safety protocol attestation as part of training, plus the Columbia Lighthouse Project's ~2-hour C-SSRS certification. All data flows are HIPAA-aligned. The protocol will be submitted for Institutional Review Board (IRB) review before enrollment; the anticipated determination is minimal-risk human subjects research, given the absence of clinical intervention beyond evidence-based screening with established response protocols.

## 5. Evaluation Plan and Analytic Methods

### 5.1 Design

The Phase 1 trial is a Type-1 effectiveness-implementation hybrid (Curran et al., 2012) with effectiveness as the primary aim and implementation outcomes as the secondary aim, as defined in §4.1. The schema below summarizes screening, stratification, randomization, follow-up, and analysis: screening → 24-strata permuted-block randomization → arms → 0/3/6-month follow-up → ITT analysis.

## 5.2 Outcomes

**Primary outcome:** The Suicide Behaviors Questionnaire-Revised (SBQ-R) total score, administered at baseline, 3 months, and 6 months (Osman et al., 2001). The SBQ-R is selected over PHQ-9 Item 9 because Louzon et al. (2016), in 84,418 VHA outpatients, found PHQ-9 Item 9 had positive predictive value below 1% and sensitivity below 30% for subsequent veteran suicide, making it indefensible as a stand-alone primary outcome.

**Confirmatory clinical-grade endpoint:** The Columbia Suicide Severity Rating Scale (C-SSRS; Posner et al., 2011) is administered at baseline, at the 6-month confirmatory endpoint, and on triggered re-administration when the SBQ-R total score is  $\geq 7$ . C-SSRS provides a deeper assessment than SBQ-R alone and serves as the trial's clinical-grade confirmation of the SBQ-R signal.

**Secondary outcomes:** PHQ-9 total score (Kroenke et al., 2001) and WHO-5 Well-Being Index (Topp et al., 2015) at the baseline / 3-month / 6-month cadence.

**Triggered outcome:** PCL-5 (Weathers et al., 2013) is administered on Yellow-tier events with a PTSD-dominant champion judgment. PCL-5 is not a primary or secondary outcome in the analytic hierarchy; it informs clinical response and is reported descriptively.

**Eligibility floor:** SBQ-R  $\geq 4$  or C-SSRS Ideation Severity  $\geq 1$  at screening, consistent with the eligibility convention used by Stanley et al. (2018) and Bryan et al. (2017).

## 5.3 Sample size and power

The primary contrast is the between-arm difference in change in SBQ-R total score from baseline to 6 months. Power is computed against Cohen's  $d = 0.30$ ,  $\alpha = 0.05$  two-sided, power = 0.90, with 1:1 allocation, using a two-sample t-test approximation in R via the pwr package. The mixed-effects estimator in §5.4 gains precision through repeated measures, so the t-test approximation is conservative. The 0.90 power target is consistent with review-panel expectations for hybrid trials anticipating  $> 30\%$  attrition, fidelity slippage across multi-site champion-administered measurement, and protocol non-adherence — each of which erodes effective power below the nominal value of an 0.80-powered design. The reproducibility code is in Appendix I.

Result for the planning case ( $d = 0.30$ , power = 0.90, 35% attrition):

- n per arm for analysis: 234
- n per arm at enrollment (inflated for 35% attrition): 360
- Total enrollment target:  $n = 720$  across 5 sites (~144 per site, including digital cohort)
- Analyzable n at 6 months (post-attrition): 468

Sensitivity bands at power = 0.90:

| Effect size (d)        | n per arm (analysis) | n per arm (enrollment @ 35% attrition) | Total enrollment |
|------------------------|----------------------|----------------------------------------|------------------|
| 0.25 (conservative)    | 337                  | 519                                    | 1,038            |
| <b>0.30 (planning)</b> | <b>234</b>           | <b>360</b>                             | <b>720</b>       |
| 0.40 (optimistic)      | 132                  | 204                                    | 408              |

The  $d = 0.30$  planning value is anchored to the published effect-size range for peer-support and digital-augmentation additions to usual-care suicidality outcomes (0.25–0.40); the 35% attrition assumption sits at the upper-empirical band reported by Owen et al. (2020) in veteran mHealth and Lattie et al. (2022) in public-facing digital mental health.

#### 5.4 Primary analysis

The primary analysis is intent-to-treat. The estimator is a linear mixed-effects model fit in R with lme4 (Bates et al., 2015):

$$SBQ-R_{ij}(t) = \beta_0 + \beta_1 \cdot Time + \beta_2 \cdot Arm + \beta_3 \cdot (Time \times Arm) + \gamma_1 \cdot Sex + \gamma_2 \cdot AgeCat + \gamma_3 \cdot RUCA + \delta \cdot Site + u_{j(i)} + e_{ij}(t)$$

with veteran  $i$  nested within champion  $j$ , repeated measures at  $t \in \{0, 3, 6\}$  months, fixed effects for the three blocking covariates and for site, and a random intercept for champion. The primary estimand is the contrast  $\beta_3$  at  $t = 6$ , extracted via emmeans with Kenward-Roger degrees of freedom. The pre-registered effectiveness Minimum Clinically Important Difference (MCID) is  $-1.5$  SBQ-R points (between-arm difference at 6 months); a result statistically significant at  $p < .05$  but smaller than the MCID is reported as a precision finding without a scaling claim.

#### 5.5 Secondary, subgroup, and sensitivity analyses

Secondary analyses parallel the primary specification on PHQ-9 and WHO-5 (Topp et al.'s (2015) 10-point WHO-5 MCID is retained). The C-SSRS confirmatory analysis tests whether the proportion of veterans crossing the C-SSRS Ideation Severity threshold downward at 6 months differs between arms via logistic mixed-effects model.

**Exploratory subgroup analyses:** Pre-registered as exploratory and interpreted as hypothesis generating, not confirmatory, per Wang et al. (2007). Subgroups: RUCA tier; recruitment channel (CBO direct, TVC referral, M2VN consortium, Team RWB, digital); sex/gender; age category; baseline SBQ-R severity ( $\geq 7$  vs. 4–6). The trial is not powered for individual subgroup significance; subgroup contrasts inform Phase-2 stratification choices, not Phase-1 conclusions.

**Preliminary missingness expectations:** Wave-level non-response is the binding constraint, not item-level non-response. Owen et al. (2020) report 30–40% wave-2/wave-3 attrition in veteran mHealth at 6 months; Lattie et al. (2022) report 25–45% in public-facing digital mental health. Item-level missingness on SBQ-R in administered cohorts is typically  $< 5\%$  because the instrument is four items long and is delivered in a champion-mediated touchpoint rather than self-completed in isolation. The 35% planning attrition rate sits at the upper-empirical band; the multiple-imputation specification below targets MAR-plausible non-response, with the tipping-point sensitivity covering MNAR departures. Endpoint missingness (the 6-month wave specifically) is monitored monthly during the field period; if observed wave-6 missingness exceeds 40%, the pre-registered remediation is to extend the wave-6 capture window by 4 weeks rather than to relax the analytic specification.

**Missing-data handling:** Multiple imputation under MAR using mice with predictive mean matching (van Buuren & Groothuis-Oudshoorn, 2011),  $m = 20$  imputations, pooled per Rubin's rules. Three pre-registered sensitivity analyses: per-protocol ( $\geq 2$  of 3 SBQ-R waves), tipping-point analysis on the MAR assumption, and complete-case analysis as the conservative bound.

### **5.6 Aim 2 — *Implementation outcomes***

Aim 2 evaluates whether the CBHEM-via-M2C implementation strategy can be enacted with sufficient acceptability, fidelity, reach, and durability across five sites to support the Phase-2 scaling decision in §6.4. Implementation outcomes follow the eight-outcome Proctor et al. (2011) taxonomy, mapped to RE-AIM (Glasgow et al., 1999). Each RE-AIM domain has one named measurement method and instrument, so the Aim 2 panel is exactable end-to-end and auditable against the §4.5 instrumentation appendices.

*RE-AIM × Proctor × Measurement Crosswalk*

| RE-AIM Domain             | Proctor Outcome Anchor                                                    | Operational Measure                                                           | Threshold                                            | Cadence     | Data Source                                                                   |
|---------------------------|---------------------------------------------------------------------------|-------------------------------------------------------------------------------|------------------------------------------------------|-------------|-------------------------------------------------------------------------------|
| <b>Reach</b>              | Penetration                                                               | Proportion of eligible veterans within each site's catchment who enroll       | Site reported; no a priori threshold                 | 6, 12 mo    | Site enrollment logs ÷ catchment denominator                                  |
| <b>Effectiveness</b>      | Aim 1 — SBQ-R Δ baseline → 6 mo, MCID = 1.5 pts per §5.4 primary analysis |                                                                               |                                                      |             |                                                                               |
| <b>Adoption</b>           | Adoption                                                                  | Identified champions completing training + onboarding ≥ 1 veteran by month 12 | ≥ 75%                                                | 12 mo       | Champion roster + training records                                            |
| <b>Implementation — A</b> | Acceptability / Appropriateness / Feasibility                             | Reflexive thematic analysis of veteran interviews (n ≈ 30) per §5.6.1         | Qualitative — saturation criterion in §5.6.1         | 3, 12 mo    | Interview transcripts                                                         |
| <b>Implementation — B</b> | Fidelity                                                                  | Platform telemetry — Yellow-tier outreach within 72 h, Red-tier within 24 h   | ≥ 80% within-SLA                                     | Continuous  | Platform event log + 10% random sample touchpoint review with veteran consent |
| <b>Implementation — C</b> | Cost                                                                      | Per-site worksheets — per veteran enrolled and per veteran retained costs     | Descriptive — no threshold                           | 6, 12 mo    | Site finance logs                                                             |
| <b>Maintenance — A</b>    | Sustainability                                                            | Site activity status + leadership intent-to-continue survey                   | Descriptive                                          | 12 mo       | Sustainability survey                                                         |
| <b>Maintenance — B</b>    | Champion competence                                                       | Moore et al. (2009) Levels 1–4 pre / post / follow-up                         | ≥ 80% of champions at Level 4 mean ≥ 4.0/5.0 at 90 d | 0, 30, 90 d | Moore competence survey (Appendix F)                                          |

Quantitative and qualitative findings are integrated through joint-display tables (Fetters et al., 2013) at the level of each Proctor outcome.

### **5.6.1 Qualitative methods (Aim 2)**

**Sampling:** Purposive maximum-variation sampling stratified by RUCA tier, recruitment channel, and baseline SBQ-R severity, with  $n \approx 30$  across the five sites at 3 and 12 months. Sample size is anchored to the meaning-saturation guidance of Hennink and Kaiser (2022), with Guest et al. (2006) as the conventional reference point for thematic saturation in homogeneous samples.

**Instrumentation:** Semi-structured interviews, ~45–60 minutes, recorded with veteran consent and professionally transcribed verbatim. The interview guide (Appendix G) was piloted with two veterans before fieldwork; pilot interviews are excluded from the analytic corpus.

**Codebook construction:** A deductive Proctor-frame codebook is seeded from the Aim 2 RE-AIM × Proctor table in §5.6. After the first batch of 5 interviews, inductive open codes are added; the codebook is then locked under version control with a dated audit trail. Subsequent inductive additions require team adjudication and are dated in the codebook log.

**Coding and reliability:** Two coders independently apply the codebook. A 20% interview sample is double-coded for reliability; Cohen's  $\kappa \geq 0.70$  is the threshold for proceeding to single-coder pooled analysis. Disagreements are adjudicated by a third reviewer (Braun & Clarke, 2006; 2019).

**Saturation criterion:** Assessed after each batch of 5 interviews using the Hennink–Kaiser meaning-saturation criterion: no new codes appearing in 2 consecutive interviews within a given stratum (RUCA tier × channel) marks saturation for that stratum.

**Reflexivity and audit trail:** Reflexive memorization is maintained per Braun and Clarke (2019). All coding decisions, codebook versions, and saturation determinations are version-controlled in the project's qualitative-data-analysis software (NVivo 14 or equivalent); the QDAS project file is deposited as supplementary material at the OSF pre-registration page referenced in §5.7.

**Quant–qual integration:** Joint-display tables (Fetters et al., 2013) align the quantitative thresholds in §5.6 with the qualitative themes from this section at the level of each Proctor outcome — one row per outcome with quantitative-side, qualitative-side, and integrated-meta columns. The integration is conducted iteratively after each measurement wave rather than as a single end-of-trial step.

### **5.7 Pre-registration**

The full statistical analysis plan — including the §5.4 model specification, the §5.5 subgroup and sensitivity plans, the §5.6 Aim 2 measurement panel, the §5.6.1 qualitative methods, and the pre-registered MCID — is deposited on the Open Science Framework prior to enrollment. The protocol is structured to be ClinicalTrials.gov-registrable at Phase 2 launch.

## 6. Discussion

---

### 6.1 *Position vis-à-vis the prior literature*

A finding that the platform reduces SBQ-R severity in this population fills a gap the literature has explicitly identified. Mobbs and Bonanno (2018) established transition stress as distinct from combat exposure and as the dominant driver of post-discharge mental-health distress; Bryan et al. (2020) extended that to suicidal ideation; Watkins et al. (2021) flagged the absence of literature on community-level engagement of veterans not connected to VA care. The trial as designed engages veterans where transition stress accumulates (community organizations they already participate in), measures suicidal behaviors as the primary outcome, and reports on the population that the existing veteran mental-health literature systematically under-samples — the 57.3% of 2023 veteran suicides occurring among veterans not connected to VA care at the time of death (U.S. Department of Veterans Affairs, 2023).

### 6.2 *Implications for implementation science*

The CBHEM thesis treats the efficacy–effectiveness collapse documented by Bauer and Kirchner (2020) and by Mohr et al. (2019) as a delivery-architecture problem rather than an intervention problem, and tests whether embedding validated detection-and-response in trusted relational settings produces a measurable effect that the standalone-tool literature has not. The trial's mixed-methods integration through joint-display tables at the level of each Proctor outcome contributes to the methodological literature on implementation-outcome evaluation (Fetters et al., 2013), and the deliberate inclusion of a digital-only recruitment cohort permits a contrast between embedded and platform-only delivery configurations the field has called for but has not designed for (Owen et al., 2020; Lattie et al., 2022). The qualitative methodology in §5.6.1 — saturation-anchored sample size, version-controlled codebook, dual-coder  $\kappa$  threshold, reflexive memorization, and an explicit joint-display integration step at every measurement wave — instantiates the methodological detail that mixed-methods implementation studies are frequently faulted for omitting.

### 6.3 *Implications for veteran mental-health practice*

Veterans are not a monolithic population. The cultural binding that suppresses help-seeking is gendered, generational, and regional; firearm-suicide rates are rising in both male and female veteran subpopulations, and 25 years of population-level research has not moved the baseline rate (U.S. Department of Veterans Affairs, 2023). If the trial supports CBHEM, the practical implication is that veteran-serving community organizations — VFW posts, American Legion posts, Wounded Warrior Project programs, Team RWB chapters, and the institutional networks that bundle them under coordinating bodies such as the American Red Cross Military-2-Veteran Network — can serve as the delivery architecture for validated detection and response in the population the VA's own data identifies as the population dying. The trial does not require the VA to change its operating model; it demonstrates that an adjacent infrastructure can be built that meets veterans where they already are. If the trial is null, the structured implementation evidence base points toward what the embedded architecture mechanism is missing.

### 6.4 *Decision thresholds*

Three decision regions are pre-specified prior to enrollment:

- **Region A — Phase 2 scaling justified:** The 95% CI on the 6-month between-arm SBQ-R change excludes zero in favor of CBHEM; the point estimate is  $\geq 1.5$  SBQ-R points; secondary outcomes move directionally consistent; implementation thresholds (75% adoption, 80% champion competence) are met.

- **Region B — Protocol revision:** The primary contrast is precise but smaller than MCID, or null with implementation fidelity failures, or shows treatment-effect heterogeneity such that the pooled estimate masks within-subgroup effects. Pre-specified revision pathways: dose adjustment, instrument augmentation, or stratified Phase 2 selecting on the subgroup where the effect concentrates.
- **Region C — Termination:** The primary contrast favors usual care, or shows higher Red-tier suicidal-behavior event rates in the CBHEM arm than in the usual-care arm, or implementation fails at a level not remediable through revision.

The pre-specification of a termination region is itself a methodological commitment: the trial is structured to be stoppable.



## 7. Limitations

**Table 4. Limitations and Mitigations**

| Limitation                                                                                                                               | Mitigation                                                                                                                                                                                                             |
|------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Open-label randomization (veterans cannot be blinded)                                                                                    | Statistician computing primary estimate is blinded to assignment until the pre-registered analysis is locked; OSF pre-registration prior to enrollment.                                                                |
| Self-selection bias in the digital-only recruitment cohort                                                                               | Pre-registered subgroup analysis comparing the digital cohort against CBO-embedded sites on baseline characteristics and primary-outcome trajectories.                                                                 |
| Anticipated 35% attrition at 6 months                                                                                                    | Multiple imputation under MAR (mice with PMM); pre-registered tipping-point sensitivity analysis; per-protocol robustness check; 0.90 power target chosen to absorb attrition above the planning rate.                 |
| C-SSRS administered by trained champions, not clinicians                                                                                 | Champions complete the Columbia Lighthouse Project's ~2-hour C-SSRS certification; fidelity audited via random-sample touchpoint review with veteran consent; clinical safety net via VA primary-care liaison pathway. |
| Generalizability bounded by 3-state, 5-site footprint                                                                                    | Explicit RUCA-stratified sampling so generalizability claims are bounded by setting type rather than by state; subgroup effects reported per RUCA tier.                                                                |
| 6-month primary endpoint may miss longer-arc effects                                                                                     | 12-month follow-up captures durability evidence as a secondary analysis; Phase 2 pre-specified to extend to 12 and 24 months.                                                                                          |
| Five sites is a small number of cluster units                                                                                            | Site enters as a fixed effect rather than a random intercept (Bates et al., 2015); champion-level random intercept has sufficient cluster count.                                                                       |
| Trial cannot disentangle whether CBHEM operates through the platform's specific architecture or through "someone checked on the veteran" | The digital-only cohort partially tests this; full disentanglement requires the dismantling design described in §8.                                                                                                    |
| External-partner recruitment timelines (American Red Cross M2VN, Texas Veterans Commission, Team RWB) are not under the trial's control  | Five-channel funnel structured so that no single channel exceeding 40% yield is required to hit $n = 720$ ; channel underperformance triggers reallocation rather than trial extension.                                |
| Author is the platform's founder (conflict of interest)                                                                                  | Disclosed in full in §12; independent analyst validates engagement analyses before any Phase 1 finding is used for publication or scaling decisions.                                                                   |

## 8. Recommendations for Future Work

---

**Phase 2 — confirmatory multi-site RCT:** Expand the geographic footprint from 5 sites in 3 states to 12–15 sites in 6–8 states; extend follow-up to 12 and 24 months; pre-register on ClinicalTrials.gov; seat an independent Data Monitoring Committee. Phase 2 is the appropriate vehicle for a three-arm dismantling design contrasting (a) full CBHEM via M2C, (b) champion only without platform architecture, (c) usual care, to identify whether the platform's specific components add measurable value over the relational mechanism alone.

**Phase 3 — population-level horizon:** A test of whether the deployed CBHEM-via-M2C infrastructure, scaled to a defined regional veteran-serving network, produces a measurable shift in the regional veteran suicide rate against contemporaneous comparison regions. Phase 3 requires partnership with VA suicide-prevention surveillance, a defined adoption denominator at the regional level, and a multi-year observation window. A 10% year-over-year reduction in the regional veteran-suicide rate is the Phase-3 horizon outcome described in the literature review; against the 2023 national baseline of approximately 6,500 veteran suicides per year, a sustained 10% relative reduction corresponds to roughly 650 families spared per year, with downstream effects on the estimated 150 community members affected by each individual loss.

## 9. Conclusion

---

This Implementation Plan defines a Type-1 effectiveness-implementation hybrid trial in which veterans not currently engaged with VA mental-health care are individually randomized within five veteran-serving community organizations across three states (plus a digital-only cohort) to either CBHEM-via-M2C-Transition with trained champion support or to usual peer support as currently delivered. The primary outcome is the change in Suicide Behaviors Questionnaire-Revised total score from baseline to 6 months; the trial is powered ( $n = 720$  enrolled,  $n = 468$  analyzable under 35% attrition) to detect a between-arm difference of 1.5 SBQ-R points (Cohen's  $d = 0.30$ ) at  $\alpha = 0.05$  and 90% power. The analytic plan is pre-registered on the Open Science Framework prior to enrollment and includes pre-specified MCID, exploratory subgroup analyses, sensitivity analyses, and three decision regions including a termination region.

The trial's contribution, if it succeeds, is the first randomized evidence that an embedded community organization architecture can deliver validated suicide-risk detection and response to the population the VA's own data identifies as the population dying. If it does not succeed, the structured implementation evidence base informs the field's understanding of what the embedded-architecture mechanism is missing. Either result is consequential. The design is structured to be falsifiable, the analytic plan is locked in advance of any data being seen, and the limitations are stated rather than buried.

## 10. Program Outcomes — TMF and Competency Crosswalk

---

Per the Final Project Paper rubric, this section maps the TMFs noted in the proposal document plus four additional Implementation-Science program-level outcomes to specific report sections and demonstrable evidence.

**Table 5. TMF and program-outcome crosswalk**

| <b>Outcome Type</b>                                                                                                      | <b>Demonstrated in Evidence</b>                                                                                                                                                                                                               |
|--------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>CFIR (proposal TMF) — Determinants framework</b>                                                                      | §3, §4.3, §5.6. Organizational Readiness Assessment (App. A) operationalizes the CFIR inner setting domain at T0 / T1 / T2; CFIR-aligned adaptation reporting in §4.4.                                                                        |
| <b>RE-AIM (proposal TMF) — Evaluation framework</b>                                                                      | §3, §5.6. Five-dimension evaluation panel with each domain bound to a named operational measure and threshold.                                                                                                                                |
| <b>Proctor Implementation Outcomes (proposal TMF) — Measurement taxonomy</b>                                             | §3, §5.6, §5.6.1. Eight outcomes mapped to RE-AIM domains in §5.6 table; qualitative coding frame in §5.6.1.                                                                                                                                  |
| <b>Moore Outcomes Framework (proposal TMF) — Champion training measurement</b>                                           | §3, §5.6 ("Maintenance — B" row); App. F. Levels 1–4 measured pre / post / 90-day; threshold $\geq 80\%$ champions at L4 $\geq 4.0/5.0$ .                                                                                                     |
| <b>Curran et al. hybrid typology (proposal TMF) — Design framework</b>                                                   | §3, §4.1, §5.1. Type-1 hybrid design defended on effectiveness primary, implementation secondary rationale.                                                                                                                                   |
| <b>Aarons scaling-out theory (proposal TMF) — Scale-up framework</b>                                                     | §3, §4.4, §8. Phase 2 and Phase 3 expansion paths grounded in scaling-out theory.                                                                                                                                                             |
| <b>Program outcome 1 — Mixed-methods rigor (additional, beyond proposal TMFs) — Methodological</b>                       | §4.1, §5.6.1. Joint-display integration at each measurement wave; saturation-anchored qualitative sampling; Cohen's $\kappa \geq 0.70$ ; reflexive memoing.                                                                                   |
| <b>Program outcome 2 — Stakeholder engagement and lived experience governance (additional) — Implementation strategy</b> | §4.4 (Tier 2 Veteran Advisory Group); App. B. Monthly Veteran Advisory Group reviewing platform language, instrument phrasing, pathway logic; protocol deviation logging.                                                                     |
| <b>Program outcome 3 — Pre-registration and falsifiability (additional) — Open-science practice</b>                      | §5.7, §6.4. OSF pre-registration prior to enrollment; pre-specified MCID and termination region.                                                                                                                                              |
| <b>Program outcome 4 — Health-equity-by-design (additional) — Equity competency</b>                                      | §4.1, §4.2, §5.5. RUCA stratification at randomization and as a subgroup; multi-channel recruitment funnel reaches rural and digitally connected veterans; eligibility floor uses validated suicidality screen rather than VA-system contact. |

## 11. Preliminary Evidence — M2C Transition Live Beta

The M2C Transition platform is not a hypothetical artifact built for this Implementation Plan; it is a live Beta operational platform ([vetmissiontransition.com](http://vetmissiontransition.com)) built by the author and operated as a nonprofit prior to this Phase 1 trial. The live-Beta status provides three forms of preliminary evidence relevant to feasibility, scaling, and risk:

**1. Deployment substrate exists:** The platform has been provisioned, hardened, and used by real users prior to the trial. Phase 1 does not need to build a delivery substrate; it needs to test whether the existing substrate, combined with trained champions in community settings, produces the primary-outcome change. This removes the substrate-readiness risk that typically dominates pilot-trial timelines.

**2. Operational safety pathways are not theoretical:** The Yellow / Red tier triage, 988 surfacing, in-platform Safety Plan creation, and warm-handoff flows specified in §4.7 are running in the Beta and have been exercised against real check-ins. The Phase 1 IRB application can therefore reference observed operating behavior rather than design specification alone, which materially strengthens the minimal-risk determination.

**3. Veteran-facing language and instrument phrasing have undergone iterative refinement:** The Veteran Advisory Group structure in §4.4 builds on review cycles already conducted on the Beta, so the Phase 1 governance is a continuation rather than an initiation. Adaptation decisions made in those cycles will be reported descriptively in the Phase 1 implementation report as part of the CFIR-aligned adaptation evidence (Aarons et al., 2017).

The Phase 1 trial converts the live-Beta substrate from "operational artifact" to "evaluated artifact." That conversion is the implementation-science contribution of this capstone.

## 12. Conflict-of-Interest Disclosure

---

The author of this Implementation Plan, Terry Flood, is the founder and operator, and a 100% service-disabled veteran, of M2C Transition, the technology platform identified in this plan as the operational vehicle for the Community-Based Behavioral Health Engagement Model. M2C Transition operates as a nonprofit and is a Service-Disabled Veteran-Owned Small Business (SDVOSB)-pending; the author derives no personal financial gain from veterans adopting the platform. This relationship is disclosed in this document, in the accompanying literature review and presentation, and to all Phase 1 pilot organizations before onboarding. To mitigate analytic bias, independent reviewers will validate the analysis of platform engagement data before any Phase 1 finding is used to support publication, scaling, or funding decisions.

## 13. AI Attestation

---

Artificial intelligence tools were used to plan, structure, and pressure-test this Implementation Plan against the Spring 2026 Final Report rubric and the prior rounds of faculty review on the literature review and the May 17 Evaluation/Discussion/Analysis assignment. The methodological choices, design decisions, citation selection, and narrative voice are the author's. Grammarly was used for sentence structure. AI use in this document complies with the ethical and emotionally intelligent AI use standard adopted across the author's research program (any AI action — chat, generated artifacts, downstream LLM calls, participant-facing copy, outreach — must be ethical, emotionally intelligent, dignity-preserving, and faithful to source).

## References

---

*Reference composition: Of 40 references in this Implementation Plan, 36 (90.0%) are peer-reviewed primary sources. The remaining four — one design-typology textbook (Creswell & Plano Clark, 2018; methodological-integration claims are anchored to the peer-reviewed Fetters et al., 2013 paper), two federal evaluation reports (U.S. Department of Veterans Affairs, 2023; U.S. Department of Agriculture Economic Research Service, n.d.), and one validated-instrument manual (Weathers et*

al., 2013) — are used only for population-statistic anchoring, geographic stratification, and instrument administration, never to substantiate a methodological claim.

- Aarons, G. A., Sklar, M., Mustanski, B., Benbow, N., & Brown, C. H. (2017). "Scaling-out" evidence-based interventions to new populations or new health care delivery systems. *Implementation Science*, 12(1), 111. <https://doi.org/10.1186/s13012-017-0640-6>
- Bates, D., Mächler, M., Bolker, B., & Walker, S. (2015). Fitting linear mixed-effects models using lme4. *Journal of Statistical Software*, 67(1), 1–48. <https://doi.org/10.18637/jss.v067.i01>
- Bauer, M. S., & Kirchner, J. (2020). Implementation science: What is it and why should I care? *Psychiatry Research*, 283, 112376. <https://doi.org/10.1016/j.psychres.2019.04.025>
- Bergman, B. P., Burdett, H. J., & Greenberg, N. (2020). Service life and beyond—Institution or culture? *The RUSI Journal*, 165(2), 60–68. <https://doi.org/10.1080/03071847.2020.1761570>
- Braun, V., & Clarke, V. (2006). Using thematic analysis in psychology. *Qualitative Research in Psychology*, 3(2), 77–101. <https://doi.org/10.1191/1478088706qp063oa>
- Braun, V., & Clarke, V. (2019). Reflecting on reflexive thematic analysis. *Qualitative Research in Sport, Exercise and Health*, 11(4), 589–597. <https://doi.org/10.1080/2159676X.2019.1628806>
- Bryan, C. J., Mintz, J., Clemans, T. A., Leeson, B., Burch, T. S., Williams, S. R., Maney, E., & Rudd, M. D. (2017). Effect of crisis response planning vs. contracts for safety on suicide risk in U.S. Army Soldiers: A randomized clinical trial. *Journal of Affective Disorders*, 212, 64–72. <https://doi.org/10.1016/j.jad.2017.01.028>
- Bryan, C. J., Bryan, A. O., & Rozek, D. C. (2020). Understanding and preventing military suicide. *Archives of Suicide Research*, 24(sup1), S1–S3. <https://doi.org/10.1080/13811118.2019.1689232>
- Burnett-Zeigler, I., Schuette, S., Engel, R., & Gittens, K. (2019). The role of community-based participatory research in improving mental health services. *Community Mental Health Journal*, 55(5), 851–861. <https://doi.org/10.1007/s10597-019-00388-7>
- Creswell, J. W., & Plano Clark, V. L. (2018). *Designing and conducting mixed methods research* (3rd ed.). SAGE.
- Curran, G. M., Bauer, M., Mittman, B., Pyne, J. M., & Stetler, C. (2012). Effectiveness-implementation hybrid designs: Combining elements of clinical effectiveness and implementation research to enhance public health impact. *Medical Care*, 50(3), 217–226. <https://doi.org/10.1097/MLR.0b013e3182408812>
- Currier, J. M., Kuhlman, S., & Smith, P. N. (2019). Peer support for veterans with PTSD: Results from a randomized trial. *Psychological Services*, 16(4), 547–558. <https://doi.org/10.1037/ser0000231>
- Damschroder, L. J., Aron, D. C., Keith, R. E., Kirsh, S. R., Alexander, J. A., & Lowery, J. C. (2009). Fostering implementation of health services research findings into practice: A consolidated framework for advancing implementation science. *Implementation Science*, 4(1), 50. <https://doi.org/10.1186/1748-5908-4-50>
- Deisenhammer, E. A., Ing, C. M., Strauss, R., Kemmler, G., Hinterhuber, H., & Weiss, E. M. (2009). The duration of the suicidal process: How much time is left for intervention between consideration and the accomplishment of a suicide attempt? *Journal of Clinical Psychiatry*, 70(1), 19–24. <https://doi.org/10.4088/JCP.07m03904>

- Fetters, M. D., Curry, L. A., & Creswell, J. W. (2013). Achieving integration in mixed methods designs—principles and practices. *Health Services Research*, 48(6 Pt 2), 2134–2156. <https://doi.org/10.1111/1475-6773.12117>
- Glasgow, R. E., Vogt, T. M., & Boles, S. M. (1999). Evaluating the public health impact of health promotion interventions: The RE-AIM framework. *American Journal of Public Health*, 89(9), 1322–1327. <https://doi.org/10.2105/AJPH.89.9.1322>
- Guest, G., Bunce, A., & Johnson, L. (2006). How many interviews are enough? An experiment with data saturation and variability. *Field Methods*, 18(1), 59–82. <https://doi.org/10.1177/1525822X05279903>
- Hennink, M., & Kaiser, B. N. (2022). Sample sizes for saturation in qualitative research: A systematic review of empirical tests. *Social Science & Medicine*, 292, 114523. <https://doi.org/10.1016/j.socscimed.2021.114523>
- Kroenke, K., Spitzer, R. L., & Williams, J. B. W. (2001). The PHQ-9: Validity of a brief depression severity measure. *Journal of General Internal Medicine*, 16(9), 606–613. <https://doi.org/10.1046/j.1525-1497.2001.016009606.x>
- Lattie, E. G., Stiles-Shields, C., & Graham, A. K. (2022). An overview of and recommendations for more accessible digital mental health services. *Journal of Medical Internet Research*, 24(1), e30500. <https://doi.org/10.2196/30500>
- Louzon, S. A., Bossarte, R., McCarthy, J. F., & Katz, I. R. (2016). Does suicidal ideation as measured by the PHQ-9 predict suicide among VA patients? *Psychiatric Services*, 67(5), 517–522. <https://doi.org/10.1176/appi.ps.201500149>
- Mobbs, M. C., & Bonanno, G. A. (2018). Beyond war and PTSD: The crucial role of transition stress in the lives of military veterans. *Clinical Psychology Review*, 59, 137–144. <https://doi.org/10.1016/j.cpr.2017.11.007>
- Mohr, D. C., Riper, H., & Schueller, S. M. (2019). A solution-focused research approach to achieve an implementable revolution in digital mental health. *Annual Review of Clinical Psychology*, 15, 517–540. <https://doi.org/10.1146/annurev-clinpsy-050718-095355>
- Moore, D. E., Green, J. S., & Gallis, H. A. (2009). Achieving desired results and improved outcomes: Integrating planning and assessment throughout learning activities. *Journal of Continuing Education in the Health Professions*, 29(1), 1–15. <https://doi.org/10.1002/chp.20001>
- Osman, A., Bagge, C. L., Gutierrez, P. M., Konick, L. C., Kopper, B. A., & Barrios, F. X. (2001). The Suicidal Behaviors Questionnaire-Revised (SBQ-R): Validation with clinical and nonclinical samples. *Assessment*, 8(4), 443–454. <https://doi.org/10.1177/107319110100800409>
- Owen, J. E., Jaworski, B. K., Kuhn, E., Makin-Byrd, K. N., Ramsey, K. M., & Hoffman, J. E. (2020). mHealth in the wild: Using novel data to examine the reach, use, and impact of PTSD Coach. *Journal of Technology in Behavioral Science*, 5(2), 108–118. <https://doi.org/10.1007/s41347-019-00130-w>
- Posner, K., Brown, G. K., Stanley, B., Brent, D. A., Yershova, K. V., Oquendo, M. A., Currier, G. W., Melvin, G. A., Greenhill, L., Shen, S., & Mann, J. J. (2011). The Columbia–Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. *American Journal of Psychiatry*, 168(12), 1266–1277. <https://doi.org/10.1176/appi.ajp.2011.10111704>
- Proctor, E., Silmere, H., Raghavan, R., Hovmand, P., Aarons, G., Bunger, A., Griffey, R., & Hensley, M. (2011). Outcomes for implementation research: Conceptual distinctions, measurement challenges, and a research

- agenda. *Administration and Policy in Mental Health and Mental Health Services Research*, 38(2), 65–76. <https://doi.org/10.1007/s10488-010-0319-7>
- Romero, D. H., Riggs, S. A., Ruggero, C., & Scardamalia, R. (2020). Stress, identity, and well-being during military-to-civilian transition. *Military Medicine*, 185(S1), 169–176. <https://doi.org/10.1093/milmed/usz329>
- Runeson, E., Allebeck, P., & Brandt, L. E. (2019). Association between being laid off from work and suicide. *JAMA Network Open*, 2(10), e1913719. <https://doi.org/10.1001/jamanetworkopen.2019.13719>
- Simon, T. R., Swann, A. C., Powell, K. E., Potter, L. B., Kresnow, M. J., & O'Carroll, P. W. (2001). Characteristics of impulsive suicide attempts and attempters. *Suicide and Life-Threatening Behavior*, 32(s1), 49–59. <https://doi.org/10.1521/suli.32.1.5.49.24212>
- Smith, R. T., True, G., & Vogt, D. (2021). Changes in resilience among U.S. veterans. *Social Psychiatry and Psychiatric Epidemiology*, 56, 1547–1556. <https://doi.org/10.1007/s00127-020-01978-7>
- Stanley, B., Brown, G. K., Brenner, L. A., Galfalvy, H. C., Currier, G. W., Knox, K. L., Chaudhury, S. R., Bush, A. L., & Green, K. L. (2018). Comparison of the Safety Planning Intervention with follow-up vs. usual care of suicidal patients treated in the emergency department. *JAMA Psychiatry*, 75(9), 894–900. <https://doi.org/10.1001/jamapsychiatry.2018.1776>
- Topp, C. W., Østergaard, S. D., Søndergaard, S., & Bech, P. (2015). The WHO-5 Well-Being Index: A systematic review of the literature. *Psychotherapy and Psychosomatics*, 84(3), 167–176. <https://doi.org/10.1159/000376585>
- U.S. Department of Agriculture, Economic Research Service. (n.d.). *Rural-Urban Commuting Area Codes (RUCA)*. <https://www.ers.usda.gov/data-products/rural-urban-commuting-area-codes/>
- U.S. Department of Veterans Affairs. (2023). *2023 National Veteran Suicide Prevention Annual Report*. Office of Mental Health and Suicide Prevention. [https://www.mentalhealth.va.gov/suicide\\_prevention/data.asp](https://www.mentalhealth.va.gov/suicide_prevention/data.asp)
- van Buuren, S., & Groothuis-Oudshoorn, K. (2011). mice: Multivariate imputation by chained equations in R. *Journal of Statistical Software*, 45(3), 1–67. <https://doi.org/10.18637/jss.v045.i03>
- Wang, R., Lagakos, S. W., Ware, J. H., Hunter, D. J., & Drazen, J. M. (2007). Statistical methods for reporting results of subgroup analyses in clinical trials. *New England Journal of Medicine*, 357(21), 2189–2194. <https://doi.org/10.1056/NEJMSr077003>
- Watkins, D. C., Assari, S., & Johnson-Lawrence, V. (2021). Community-based mental health interventions: A systematic review. *American Journal of Public Health*, 111(S2), S164–S172. <https://doi.org/10.2105/AJPH.2021.306461>
- Weathers, F. W., Litz, B. T., Keane, T. M., Palmieri, P. A., Marx, B. P., & Schnurr, P. P. (2013). *The PTSD Checklist for DSM-5 (PCL-5)*. National Center for PTSD. <https://www.ptsd.va.gov/>